

Urodynamics is a group of tests that measure how well your bladder stores and releases urine. Thin, soft tubes (catheters) measure pressure while your bladder is slowly filled and then emptied. The goal is to recreate your symptoms in the office, so your team can find the cause and plan the right treatment.

About This Test

Urodynamics is not one test but several done together. Depending on your needs, it may include:

- A **flow test**: you urinate into a special toilet that measures your stream. Then the team checks how much urine is left behind.
- A **filling study**: the bladder is slowly filled while pressure is measured and you say what you feel.
- A **pressure-flow study**: pressure is measured while you urinate.
- Sometimes **X-ray pictures** are taken at the same time (video urodynamics).

Two thin tubes are used. One goes in the bladder through the urethra. A second, very small one goes in the rectum or vagina to measure belly pressure. This lets the team tell bladder pressure apart from straining.

Is It Safe?

Yes. It is a low-risk test, and there is no surgery. The main risks come from the catheters: brief burning, a small chance of a urine infection and, rarely, trouble urinating afterward. Most people rate the discomfort as low and say they would have the test again. Most people do not need an antibiotic. Some do, for example people with nerve-related bladder problems or a weak immune system.

Learn the Terms

Urodynamics

A set of tests that measure how the bladder stores and empties urine.

Bladder

The muscular sac that stores urine until you urinate.

Catheter

A thin, soft tube used to fill the bladder and measure pressure.

Filling study

Measures pressure and feeling as the bladder slowly fills.

Pressure-flow study

Measures pressure while you urinate, to see how the bladder empties.

Flow test

Measures the speed and pattern of your urine stream.

Video urodynamics

The same test with X-ray pictures added.

Post-void residual

The amount of urine left in the bladder after you urinate.

Will it hurt? Most people feel pressure and some embarrassment rather than real pain. You will be asked to cough, change position and urinate during the test. The team keeps you covered and works at your pace. It often takes about 30–60 minutes, and you can go straight home.

How to Get Ready

- You may be asked to arrive with a **comfortably full bladder** and not to urinate just before, because the test starts with you urinating. Eat and drink as usual unless told otherwise.
- If you **self-catheterize**, ask whether to keep your usual schedule on the test day.
- Take your **usual medicines**, but ask whether to **stop any bladder medicines** first, since they can change the results.
- No sedation is needed, so you can usually **drive yourself home**.
- You may be asked for a **urine sample** to check for infection.

Tell your team ahead of time if you:

- Have signs of a **urine infection** (burning or fever). The test may need to be moved.
- Have a **spinal cord injury**. Your blood pressure will be watched during the test.
- Have had a narrowing of the urethra or surgery on it, or have an artificial urinary sphincter
- Take a blood thinner, or have any allergies (including latex)

What Happens During the Test

- 1 You urinate into a special toilet, and the team checks how much is left behind.
- 2 Thin catheters are gently placed: one in the bladder and a small one in the rectum or vagina.
- 3 The bladder is slowly filled. You say when you first feel it and when you feel a strong urge.
- 4 You urinate while the pressures are recorded. If you cannot go in the test room, that is still useful information. Then the catheters come out.

After the Test

- You can go back to **normal activities right away**.
- Mild **burning when you urinate** for a day or so is common. Drinking extra water helps.
- A **pink tinge** in the urine for a short time is common and usually clears within a day.

Call the office that did your test if you have:

- A temperature of **100.4 °F (38 °C)** or higher, or chills
- Burning or pain that gets worse instead of better
- A weak stream, or you feel you are not emptying

Go to an emergency department if:

- You cannot urinate for several hours and your bladder feels full or painful
- Your urine looks like red wine or has clots
- You cannot reach the office and are worried

Three Things to Remember

1. Urodynamics shows how your bladder fills and empties, to guide treatment.
2. Ask whether to stop any bladder medicines before the test.
3. Expect pressure more than pain. You go home right away.